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PHARMACY AND THERAPEUTICS BULLETIN

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| SYNTHETIC DOCUMENT - NOT REAL MEDICAL GUIDANCE. Fabricated for a
| software demonstration. Renuvex is an invented medication. Nothing here
| describes a real product or should inform any clinical decision.

1. Scope of this bulletin

This bulletin consolidates three unrelated items arising from the January meeting of the Pharmacy and Therapeutics Committee. Items are presented in the order discussed and are not ranked by clinical significance. Sites should read section 3 in full.

2. Formulary status

Renuvex remains a restricted-formulary agent at all sites. The restriction is administrative and relates to acquisition cost; it carries no safety implication. Requests for non-formulary use continue to be handled by the site pharmacy lead.

3. Renal considerations in continuing therapy

Renuvex is eliminated by the kidney and its elimination half-life lengthens as glomerular filtration falls. The Committee reviewed twenty-two months of dispensing and laboratory data from the three participating sites and found considerable variation in how often renal function was being reassessed after therapy was established - in a small number of instances, not at all. Having considered that variation, and noting that a change in renal function is not reliably apparent from symptoms, the Committee agreed that patients on Renuvex who have chronic kidney disease should have serum creatinine measured quarterly while treatment continues, and that sites should put arrangements in place to make that happen rather than relying on opportunistic testing. The requirement is effective from the date of this bulletin. Where a site cannot meet it from existing capacity, the site pharmacy lead should escalate to the Committee rather than allow the interval to extend.

Separately, and not as a monitoring requirement, the Committee noted that baseline renal function is often recorded at first prescription. That practice is encouraged but is not the subject of any interval requirement.

4. Supply

A national supply constraint affecting the 20 mg presentation has been resolved. No action is required.

5. Contact

Queries relating to this bulletin should be directed to the Committee secretary.

- End of bulletin -